INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin + Oral Etoposide

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00319
Version 5

Treatment stages

01 **Day 1 infusion and Days 1–3 oral chemotherapy**
One infusion day plus three capsule days

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Carboplatin + Oral Etoposide · National Cancer Control Programme (NCCP) 00319

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your unit urgently for fever at its stated threshold, shaking chills, sudden illness, new or worsening breathlessness, chest pain, unusual or uncontrolled bleeding, severe diarrhoea, repeated vomiting, inability to drink, very little urine, or a spreading/blistering rash. Swelling of the tongue/throat or trouble breathing can be an emergency. Report new tinnitus, hearing change, numbness or difficulty walking promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your two chemotherapy medicines

This chemotherapy combination aims to control extensive-disease small-cell lung cancer, ease symptoms and help people live longer. Expected benefit depends on the cancer, general health and treatment choices. Ask what the team hopes it will achieve for you.

PLATINUM CHEMOTHERAPY

Carboplatin



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) through platinum links

Carboplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing, but healthy tissues may also be affected. The dose is tailored using kidney function because the kidneys clear carboplatin.

Platinum reaches cells → DNA links form → Cell division disrupted

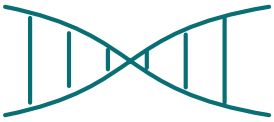
What to watch for

Report ringing in the ears, hearing changes, numbness or loss of function. Report infection or unusual bruising/bleeding promptly and describe any symptoms during the infusion.

TOPOISOMERASE-II INHIBITOR CHEMOTHERAPY

Oral etoposide

Capsules; supplied brand may vary



Interferes with DNA repair and cell division

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Capsules absorbed → DNA repair disrupted → Cell growth slowed

What to watch for

Follow the exact capsule schedule and record doses. Report missed/vomited doses, infection, bruising, mouth ulcers or severe bowel symptoms. Do not double or convert doses yourself.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Evidence and its limits

Ask which research best matches your treatment and goals.

The studies cited by NCCP support platinum–etoposide treatment and provide background on using etoposide by mouth, but they did not test this exact carboplatin plus all-oral three-day schedule. The oral comparison used cisplatin and a mixture of intravenous and oral etoposide. A separate pooled analysis compared platinum choices across several trials. These results cannot give a precise survival estimate for this prescription or predict your individual benefit.

SACTCHECK · CHAPTER 02

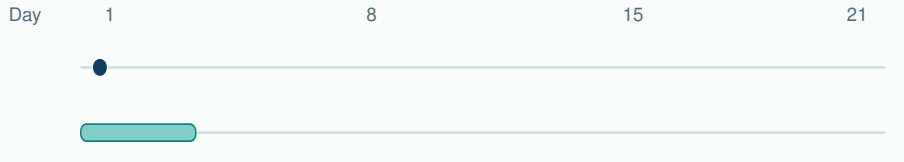
Your schedule, tests and appointments

Carboplatin + Oral Etoposide · NCCP 00319

Day 1 infusion and Days 1–3 capsules in a 21-day cycle

Carboplatin + Oral Etoposide

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE INFUSION DAY PLUS THREE CAPSULE DAYS

Day 1 infusion and Days 1–3 oral chemotherapy

D1 | D2–D3 | D4–D20 | D21

Carboplatin is planned into a vein on Day 1, before oral etoposide. Etoposide capsules are prescribed twice daily on Days 1, 2 and 3 only, on an empty stomach. The cycle repeats every 21 days while treatment is helping and side effects remain acceptable. Your team decides when to delay, change or stop; the source does not set a fixed number of cycles.

Carboplatin is planned into a vein on Day 1, before oral etoposide. Etoposide capsules are prescribed twice daily on Days 1, 2 and 3 only, on an empty stomach. The cycle repeats every 21 days while treatment is helping and side effects remain acceptable. Your team decides when to delay, change or stop; the source does not set a fixed number of cycles.

BASELINE → TREATMENT → FOLLOW-UP

Checks across treatment

These columns show order of care, not equal time intervals. Blood counts are arranged weekly before treatment; kidney/liver tests repeat before each cycle. Scans and extra tests follow the consultant plan. There is no fixed universal scan interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00319 v5 · official regimen

THE WHOLE TREATMENT JOURNEY

Carboplatin + Oral Etoposide: treatment and checks

Extensive-disease small-cell lung cancer · carboplatin + oral etoposide

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Day 1	Days 2–3	Recovery / next cycle	Course review
Treatment and oral doses NCCP 00319	Confirm extensive small-cell lung cancer (SCLC) and cisplatin unsuitability	Carboplatin infusion before oral etoposide	Oral etoposide twice daily on both days	No further etoposide until next authorised cycle	Continue only while benefit/ tolerability support it
 **Blood counts** NCCP 00319	Baseline blood profile	Review pretreatment results	Extra review if illness or bleeding	full blood count (FBC) weekly prior to treatment, as arranged	Follow any persistent low counts
Kidneys, liver and albumin NCCP 00319	Renal/liver profile; measured GFR preferred	Verify kidney-based carboplatin dose	Report poor intake or low urine	Renal/liver profile before each cycle	Reassess after illness or organ change
Hearing and nerves NCCP 00319	Baseline symptoms; audiology if indicated	Report new changes	Report tinnitus, numbness or functional problems	Regular hearing/ neurological review	Follow symptoms that persist
Prevention, adherence and safety NCCP 00319	Pharmacy capsule teaching; contact number	Dedicated carboplatin antiemetics; capsule sequence	Record capsules; follow reconciled antiemetics	Support plan covers two days after last systemic anticancer therapy (SACT); report missed/ vomited doses	Review medicines and unused capsule return
 **Disease assessment** NCCP 00319	Confirm treatment aim	Clinical review	Report significant new symptoms	Tests/scans according to Consultant plan	Agree continuation, change or stop

What each check is for

1 **Before the first cycle**

Before treatment authorisation

Confirm disease, organ function and safe home dosing.

Ask / act: Confirm consent, capsule teaching and prevention plan.

2 **After Day 3**

After the last prescribed capsule doses

Avoid unintended continuation or replacement.

Ask / act: Record doses and follow support plan; contact team about omissions/vomiting.

3 **Before each new cycle**

After recovery and review

Counts, organ function and tolerability may change.

Ask / act: Start another capsule course only after authorisation.

4



Course review

According to Consultant plan

Treatment should continue only while appropriate.

Ask / act: Discuss benefit, toxicity and next plan; return unused capsules.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before the visit

Confirm blood-test arrangements, transport, capsule prescription and the 24-hour contact number. Tell the team if you are unwell before travelling.

STEP 2

Time in the unit

Carboplatin runs into a vein over about 30 minutes on Day 1. Etoposide is taken as capsules by mouth twice daily on Days 1–3, with the Day 1 sequence confirmed by the unit. The visit includes blood-result review, prevention medicines, preparation and observation. The pharmacy label gives your exact capsule strengths, number and times; do not calculate capsules from this guide.



STEP 3

Access for the infusion

The team selects a cannula or appropriate existing central line for carboplatin. Etoposide is taken by mouth, so this regimen does not normally require a take-home infusion pump.

STEP 4

Taking capsules home

Follow the prescribed capsule count and twice-daily Days 1–3 schedule exactly, using the written calendar to record each dose. Take capsules on an empty stomach with water; ask the pharmacist to explain the timing around food. Do not chew, open or crush capsules. If swallowing is difficult, contact the team before taking them; do not substitute an intravenous preparation or change the dose yourself.

STEP 5

Home support

Keep capsules in their original pack, away from children and excess heat or damp, following the supplied label. The manufacturer leaflet advises gloves for handling and washing hands afterwards. Do not open a blister with a leaking capsule; ask the pharmacist what to do. Return unused capsules through the pharmacy; do not put them in household waste or drains.



STEP 6

Bring to review

Bring the capsule dose record, current medicines list and symptom notes. Ask for help if remembering doses or managing the pack is difficult.

Symptoms and supportive care

Carboplatin + Oral Etoposide · NCCP 00319

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Carboplatin infusion	 **Day 1** Day 1 after clinical review
Etoposide capsules	 **Days 1–3** Twice daily on Days 1–3 as prescribed
Prevention and rescue plan	 **Days 1–5** Team reconciles support for each treatment day and two days afterwards; specific medicines have different stop days
Symptoms can occur	 **Days 1–21** Report symptoms throughout the cycle; this bar does not measure risk
Next cycle review	 **Days 20–21** Blood tests and clinical review before restarting; appointment timing may differ

Before starting: Confirm tests, exact capsule directions, prevention medicines and urgent contact arrangements.

After Day 3: Etoposide capsules stop after the prescribed three days. Prevention medicines may continue according to the written plan.

Days 1–3: Day 1 includes carboplatin and the first oral doses; Days 2–3 use oral etoposide only. Do not substitute another etoposide schedule.

Across the course: Attend blood checks and review hearing, nerves and kidney function. The number of cycles depends on benefit and tolerability; this timeline does not set a fixed course.

Why these bands stay broad: This shows one planned cycle. It does not predict when symptoms will occur or guarantee safe days. Use the prescribed calendar and team instructions if dates change. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00319 v5 · official regimen ↗

There are no guaranteed "safe days". Illness can be serious at any point in the cycle. Follow the unit's temperature threshold and emergency instructions; call for shaking chills or sudden illness even without a high temperature.

Between-treatment notes

Day 1: Infusion and first capsule day

Carboplatin runs into a vein over about 30 minutes on Day 1. Etoposide is taken as capsules by mouth twice daily on Days 1–3, with the Day 1 sequence confirmed by the unit. The visit includes blood-result review, prevention medicines, preparation and observation. The pharmacy label gives your exact capsule strengths, number and times; do not calculate capsules from this guide. Confirm the capsule calendar, prevention plan and 24-hour number before leaving.

Day 2–3: Continue prescribed capsules

Follow the prescribed capsule count and twice-daily Days 1–3 schedule exactly, using the written calendar to record each dose. Take capsules on an empty stomach with water; ask the pharmacist to explain the timing around food. Do not chew, open or crush capsules. If swallowing is difficult, contact the team before taking them; do not substitute an intravenous preparation or change the dose yourself. Record each dose. Contact the team about a missed or vomited dose; do not take an extra dose yourself.

Day 4–5: Capsules finish; support may continue

Do not continue etoposide beyond the three prescribed days unless the treating team has changed the plan. Follow the written anti-sickness schedule; its medicines do not all stop together. Call if sickness prevents food, fluids or prescribed medicines staying down.

Day 6–20: Recovery and monitoring

Attend the blood tests requested by the team. Low counts, fatigue and mouth or bowel problems may develop between visits. Hearing and nerve symptoms can build across cycles. Use the emergency contact plan at any time; do not wait for the next infusion.

Day 21: Review before another cycle

The team checks blood counts, kidney/liver results, symptoms and treatment benefit before authorising another cycle. Do not start the next capsule course until your team confirms it. Bring the medicine list and dose record.

Support around the medicines

1 Your capsule prescription

2 Missed or vomited doses

If a dose is missed, do not double the next dose or extend the course. The manufacturer leaflet advises taking the next dose when due; inform your oncology pharmacist/team and follow your written plan. If you vomit after a dose, contact the team for instructions rather than automatically taking replacement capsules. BC Cancer specifically advises no repeat dose after vomiting within one hour unless the healthcare team directs it. Persistent vomiting or inability to keep fluids down needs urgent advice.

3 Sickness prevention across several days

Carboplatin Day 1 has high sickness risk; the oral etoposide-only days have a different risk. Your team provides one clear prevention and rescue plan covering treatment days and the next two days. Some medicines overlap; take only the reconciled prescription and do not repeat the full Day 1 plan on your own.

4 Fluids and symptom reporting

Ask for a drinking plan appropriate to your heart and kidney health. Report inability to drink or reduced urine promptly. Keep mouth-care advice and the emergency number nearby.

5 Other medicines and vaccines

Show the team all prescribed and non-prescribed medicines, supplements and vaccines. Etoposide has important interactions. Check before adding or stopping medicines; ask about contraception and fertility before treatment.

Why this treatment is being offered

Start with the individual aim and the exact prescription.

The precise regimen

This guide covers carboplatin plus oral etoposide for extensive-disease small-cell lung cancer in people unsuitable for cisplatin-based treatment. It does not cover an intravenous etoposide schedule or an added immunotherapy medicine. If the prescription differs, ask for its matching guide and consent discussion.

Preparing for the first cycle

Bring concerns about medicines, swallowing, memory or home support to the team.

Baseline checks

Blood, kidney and liver tests are needed before treatment. Kidney measurements help set carboplatin dosing; hearing tests may be arranged if needed. Tell the team about existing hearing or nerve problems.

Fertility, pregnancy and breastfeeding

Discuss fertility preservation, contraception and any pregnancy plans before starting. Pregnancy and breastfeeding are excluded in this regimen. Your team explains the medicine-specific contraception period.

Taking chemotherapy at home

An oral medicine still requires careful treatment checks.

What to watch for

Tell the team early so symptoms can be assessed and treated.

Blood counts and infection

Both medicines can lower blood counts, increasing infection, bleeding or anaemia risk. Attend monitoring and report sudden illness, bruising or bleeding.

Mouth, stomach and energy

Sickness, diarrhoea, constipation, mouth soreness, appetite loss, fatigue and hair loss may occur. Ask for help early; inability to drink or persistent vomiting needs urgent advice.

Hearing, nerves and kidneys

Tell the team about tinnitus, hearing change, numbness, weakness, balance problems or reduced urine. Some effects can persist after treatment.

Less common serious effects

Serious allergic or skin reactions can occur. Etoposide also carries a risk of secondary leukaemia; ask the team to explain serious and lasting risks in your situation.

When to get urgent help

Use the unit’s 24-hour number and emergency instructions.

What to say when you call

Give the regimen name, last infusion and capsule dose, symptoms and when they began. Explain whether you can drink and pass urine. Do not delay calling to collect all the details.

Reviewing the course and next steps

The number of cycles is a clinical decision.

Before another cycle

Attend weekly blood counts as arranged and kidney/liver tests before each cycle. The team reviews toxicity and benefit; do not restart capsules until authorised.

After a change or stop

Confirm which medicines stop or continue, how unused capsules are returned, and follow-up for ongoing symptoms. Ask who will contact you about the next treatment or supportive-care plan.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber’s review. The patient page explains the checks and does not authorise treatment.

Are oral capsules a weaker form of chemotherapy?

No. Etoposide capsules are chemotherapy and can cause serious effects. Their absorption differs from an infusion, so only the treating team can choose or change the dose.

Do I continue capsules every day until the next infusion?

No. This regimen prescribes twice-daily etoposide on Days 1–3 only. Use your own written calendar and wait for the team to confirm the next cycle.

Will I always have a fixed number of cycles?

The current NCCP source continues this regimen until disease progression or unacceptable toxicity, with ongoing review. Your team agrees the number of cycles and may stop or change earlier.

> **Try explaining the plan back:** Can you describe which medicine is given through a drip, which capsules you take twice daily on Days 1–3, what to do if a dose is missed or vomited, and whom to contact urgently?

Which treatment this guide covers This guide covers carboplatin plus oral etoposide for extensive-disease small-cell lung cancer in people unsuitable for cisplatin-based treatment. It does not cover an intravenous etoposide schedule or an added immunotherapy medicine. If the prescription differs, ask for its matching guide and consent discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin AUC5 plus oral etoposide

Active medicines: carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V8 row This is the dedicated carboplatin row. Reconcile with the full multi-day plan; no duplicate steroid courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.

Additional phase-specific support

- Dedicated carboplatinAUC≥4NCISV8row; routine olanzapine is not included in this row.
- Only Day 1contains high-risk carboplatin. Days 2–3oraletop are minimal–low risk and need their own daily review within one reconciled plan.
- NCIS multi-day principle:appropriate prevention on each treatment day and two days afterwards. Define the Day 4–5/rescue plan without inventing extension of every high-risk medicine.

- Metoclopramide row does not define end day or PRN qualifier; prescriber/pharmacy must set duration and suitability.
- Check QT risk for ondansetron and all interactions; local policy and individual prescriptions apply.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 2–3 · oral etoposide, then two-day post-treatment support

Active medicines: etoposide

Emetogenic risk: minimal to low

Medicine	Dose / route	When
Continue the reconciled Day 1 prevention plan	As individually prescribed	Includes the Day 2–3aprepitant and Day 2–4dexamethasone from the carboplatin reference row where prescribed Do not repeat Day 1doses or add a second high-risk course.
Oral-etoposide-day prevention / rescue	Individualised to daily risk, symptoms and existing medicines	Before oral treatment on Days 2–3 and appropriate support for two days after finalSACT (Days 4–5) NCCP 00319 calls oral etoposide minimal–low. NCISV8p5provides a daily-risk principle, not an explicit universal Day 5drug/dose; pharmacy must reconcile it.

Additional phase-specific support

- Prescribe one clear multi-day schedule with stop days.
- No automatic routine olanzapine, prolonged aprepitant, or copied five-day cisplatin schedule.
- Patient records exact oral doses and contacts team about missed/vomited doses.
- Regular premedication is not usually required unless previous hypersensitivity; no additional routine supportive medicines specified by00319.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Ears / hearing

Carboplatin

Report tinnitus or hearing change promptly.

3 Chest / breathing

Both medicines

New breathlessness, chest pain or allergic swelling needs urgent assessment.

5 Stomach / bowel

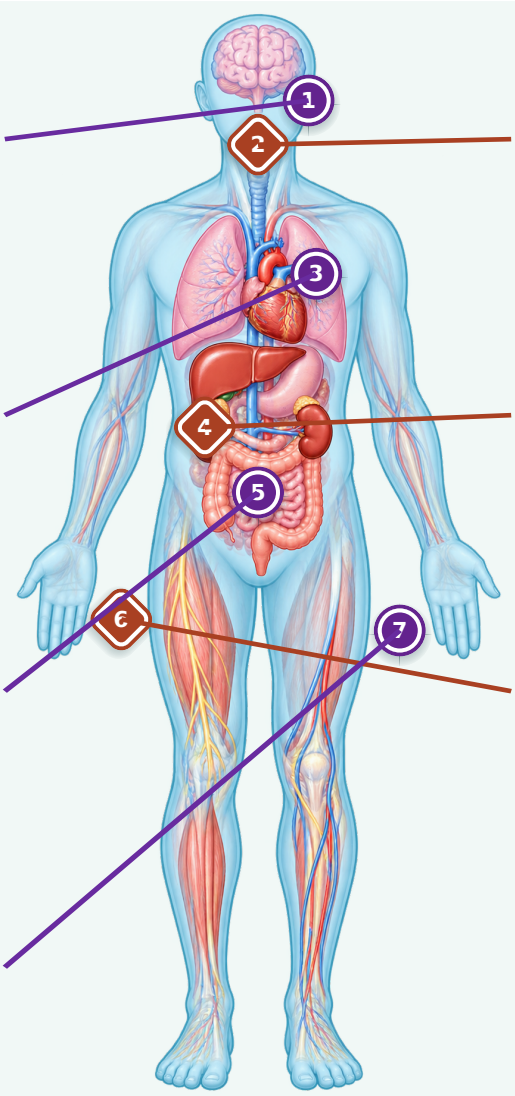
Both medicines

Use the prescribed sickness plan; call about persistent vomiting or diarrhoea.

7 Skin / blood counts

Both medicines

Report severe rash, fever, bruising or bleeding.



2 Mouth

Both medicines

Report ulcers or painful swallowing; poor intake can need urgent support.

4 Kidneys / fluids

Carboplatin

Report reduced urine or inability to drink; kidney tests guide dosing.

6 Hands / feet

Carboplatin

Report tingling, numbness, weakness or balance problems.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your unit urgently for fever at its stated threshold, shaking chills, sudden illness, new or worsening breathlessness, chest pain, unusual or uncontrolled bleeding, severe diarrhoea, repeated vomiting, inability to drink, very little urine, or a spreading/blistering rash. Swelling of the tongue/throat or trouble breathing can be an emergency. Report new tinnitus, hearing change, numbness or difficulty walking promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Carboplatin + Oral Etoposide · National Cancer Control Programme (NCCP) 00319

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00319 v5 · official regimen**
   https://healthservice.hse.ie/documents/6920/319_V5_CarboplatinEtoposide_21days.pdf
2. **Etoposide capsules manufacturer patient leaflet, January2024**
   https://www.medicines.org.uk/emc/files/pil.10534.pdf
3. **BC Cancer: oral etoposide patient information (vomited-dose advice)**
   https://www.bccancer.bc.ca/drug-database-site/Drug%20Index/Etoposide_oral_handout.pdf
4. **Carboplatin manufacturer product information: mechanism and safety**
   https://www.medicines.org.uk/emc/product/3787/smpc
5. **Etoposide capsules: manufacturer product information (Neon Healthcare)**
   https://www.medicines.org.uk/emc/product/10534/smpc
6. **Macmillan: etoposide capsules swallowing and handling advice**
   https://www.macmillan.org.uk/cancer-information-and-support/treatments-and-drugs/etoposide
7. **HSE: chemotherapy side effects and urgent contact advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NCCP NCIS Medical Oncology antiemetics V8, Table1 and multi-day note**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
9. **Johnson1991 randomised cisplatin with intravenous (IV) versus mixed IV/oral etoposide**
   https://pubmed.ncbi.nlm.nih.gov/1845847/
10. **Rossi2012 COCIS individual-patient meta-analysis: platinum choice in small-cell lung cancer (SCLC)**
    https://pubmed.ncbi.nlm.nih.gov/22473169/